



Republic of the Philippines
Department of Health
OFFICE OF THE SECRETARY

MAR 11 2024

DEPARTMENT ORDER

No. 2024 - 0081

SUBJECT: Guidelines on the Sub-allotment and Utilization of Funds to Centers for Health Development (CHDs) for the Hiring of Sixty-Three (63) Computer Programmers II and Ninety-Seven (97) Computer Programmer I

I. RATIONALE

The 8-Point Action Agenda was developed with inputs from stakeholders across various sectors in recognition of the importance of other determinants of health. It is designed with emphasis on actions to fast-track health sector improvements, focusing on the tangible outcomes for every Filipino, every community, and every healthcare worker and health institution.

One of the key strategies Para sa Bawat Pilipino (For Every Filipino) is "Teknolohiya para sa mabilis na serbisyo" that utilizes digital health and technology to enhance the efficiency and accessibility of health services. Harnessing technological advancements will streamline processes, reduce wait times, and enhance the accessibility of health services for everyone, especially those residing in remote or underserved areas. To realize this, DOH-Knowledge Management and Information Technology Service (KMITS) shall strengthen its operations through its regional counterparts with the following Information Communications Technology delivery, support and service priority action lines: (1) mandate the use of an interoperable health information system (iHIS)/electronic medical records deployment across all health facilities; (2) deploy Department of Health (DOH) developed and maintained information systems, applications, and services; (3) adopt and routine use of telemedicine among health care providers; (4) establish robust cybersecurity measures and data privacy protocols; and (5) efficient maintenance of ICT infrastructure of the CHDs.

II. OBJECTIVE

This Order aims to provide guidelines in the release, utilization and reporting of funds relative to the provision of support to CHDs for the hiring of sixty-three (63) Computer Programmer (CP) II and ninety-seven (97) CP I.

III. GENERAL GUIDELINES

The DOH through KMITS shall sub-allot funds from the MOOE allotment in the amount of Seventy-Three Million Seven Hundred Seventy Three Thousand One Hundred Ninety Three and Sixty Centavos (Php 73,773,193.60) charged to HIT funds, Fiscal Year 2024 General Appropriations Act, Republic Act 11936. ⁷⁵ +

IV. SPECIFIC GUIDELINES

1. The Director of the CHD and the Regional Office eHealth Technical Support Team shall

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be responsible for the overall implementation and monitoring of sub-allotted funds, and ensure that the unit/person-in-charge of managing the said funds and the Budget/Accounting Divisions are properly informed and compliant with these guidelines as stated in Annex A (Breakdown of Sub-allotment).

2. The roles and responsibilities of CP II and CP I are indicated in Annex B. The hired CP II and CP I at the CHDs shall be assigned to the Regional Office eHealth Technical Support Team (ROeHTST)/ Information and Communication Technology Unit (ICTU).
3. The monthly wage of each CP II is Salary Grade 15 amounting to Php 35,097.00, with up to 20% premium ~~amounting to Php 7,019.40~~. Meanwhile CP I is Salary Grade 11 each amounting to Php 25,439.00, with up to 20% premium ~~amounting to Php 5,087.80~~.
4. Monitoring expenses for the provision of technical support including the traveling expenses and other allowable expenses shall also be charged to the sub-allotted funds.
5. In the event that a portion of the sub-allotment is not fully used for the intended purposes, the remaining funds may be used to fund activities such as but not limited to training, monitoring activities, and procurement of ICT supplies relative to the implementation of DOH deployed and/or maintained information systems.
6. The Fund Utilization shall be recorded in the appropriate *Registries of Allotments, Obligations and Balances* (PS/MOOE/CO) and shall be submitted through *Statement of Appropriations, Allotment, Obligations, Balances and Disbursement* (SAAOBD) to the Financial and Management Service on or before the 10th day of the following month.
7. The sub-allotted funds shall be fully obligated and utilized by 31 December 2024.
8. Expenditures shall follow the usual budgeting, accounting and auditing rules and regulations.

V. EFFECTIVITY

This Order shall take effect immediately.

By Authority of the Secretary of Health:


MA. CAROLINA VIDAL-TAINO, CPA, MGM, CESO I
Undersecretary of Health
Management Support Cluster

Annex A. Breakdown of Sub-allotment

CHD	Designation	No. of CP	Monthly Salary (in Php) computed in 3rd Tranche as per DBM NBC 591 ¹	20% Premium	Total	No. of Months	Total Amount	Traveling expenses and other allowable expenses	TOTAL
Central Luzon Center for Health Development	CP I	7	25,439.00	5,087.80	213,687.60	12	2,564,251.20		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,480,441.60
Ilocos Center for Health Development	CP I	7	25,439.00	5,087.80	213,687.60	12	2,564,251.20		
	CP II	1	35,097.00	7,019.40	42,116.40	12	505,396.80	400,000.00	3,469,648.00
Cagayan Valley Center for Health Development	CP I	4	25,439.00	5,087.80	122,107.20	12	1,465,286.40		
	CP II	4	35,097.00	7,019.40	168,465.60	12	2,021,587.20	400,000.00	3,886,873.60
CALABARZON Center for Health Development	CP I	5	25,439.00	5,087.80	152,634.00	12	1,831,608.00		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	3,747,798.40
MIMAROPA Center for Health Development	CP I	1	25,439.00	5,087.80	30,526.80	12	366,321.60		
	CP II	8	35,097.00	7,019.40	336,931.20	12	4,043,174.40	400,000.00	4,809,496.00
Bicol Center for Health Development	CP I	6	25,439.00	5,087.80	183,160.80	12	2,197,929.60		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,114,120.00
Western Visayas Center for Health Development	CP I	6	25,439.00	5,087.80	183,160.80	12	2,197,929.60		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,114,120.00
Central Visayas Center for Health Development	CP I	8	25,439.00	5,087.80	244,214.40	12	2,930,572.80		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,846,763.20
Eastern Visayas Center for Health Development	CP I	6	25,439.00	5,087.80	183,160.80	12	2,197,929.60		
	CP II	5	35,097.00	7,019.40	210,582.00	12	2,526,984.00	400,000.00	5,124,913.60
Zamboanga Peninsula Center for Health Development	CP I	6	25,439.00	5,087.80	183,160.80	12	2,197,929.60		
	CP II	2	35,097.00	7,019.40	84,232.80	12	1,010,793.60	400,000.00	3,608,723.20
Northern Mindanao Center for Health Development	CP I	7	25,439.00	5,087.80	213,687.60	12	2,564,251.20		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,480,441.60
Davao Center for Health Development	CP I	9	25,439.00	5,087.80	274,741.20	12	3,296,894.40		
	CP II	1	35,097.00	7,019.40	42,116.40	12	505,396.80	400,000.00	4,202,291.20
SOCCSKSARGEN Center for Health Development	CP I	6	25,439.00	5,087.80	183,160.80	12	2,197,929.60		
	CP II	3	35,097.00	7,019.40	126,349.20	12	1,516,190.40	400,000.00	4,114,120.00
Cordillera Center for Health Development	CP I	2	25,439.00	5,087.80	61,053.60	12	732,643.20		
	CP II	7	35,097.00	7,019.40	294,814.80	12	3,537,777.60	400,000.00	4,670,420.80

¹ Department of Budget Management National Budget Circular 591: "Implementation of the Fourth Tranche of the Modified Salary Schedule for Civilian Personnel in the National Government Pursuant to Republic Act (RA) No. 11466"

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Caraga Center for Health Development	CP II	8	35,097.00	7,019.40	336,931.20	12	4,043,174.40	400,000.00	4,443,174.40
Metro Manila Center for Health Development	CP I	17	25,439.00	5,087.80	518,955.60	12	6,227,467.20	400,000.00	9,659,848.00
	CP II	6	35,097.00	7,019.40	252,698.40	12	3,032,380.80		
Grand Total		160							73,773,193.60


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Annex B. Roles and Responsibility of CP II and CP I

1. All hired CP II covered under these guidelines shall have the following roles and responsibilities:
 - a. Assist the Computer Maintenance Technologist (CMT) in overseeing the deployment and generation of monthly service statistics/status reports on iClinicSys, iHOMIS, DOH developed information systems, applications, and services (Annex C)
 - b. Support the CMT by monitoring key indicators outlined in the Philippine Development Plan 2023-2028 and National Objectives for Health 2023-2028 related to Information Communications Technology that are embedded into the monitoring toolkits specified in Department Circular 2022-0274, titled "Local Health Systems Maturity Level Monitoring Toolkit for P/CWHS Characteristic on Info1: Information Management System." In preparation of the eventual integration of Province/City Wide Health Systems monitoring shall extend the beyond the Universal Health Care Integration Sites (UHC IS), to include all provinces and cities within their designated region: Specifically they shall accomplish and submit the following:
 - i. Monitor and maintain a database for the implemented functional electronic medical records systems (EMRs) in the region; and submission of report to KMITS on a monthly basis (Annex C.1 and C.2)
 - ii. Monitor the health facilities implementing telemedicine and data collected in the region; and submission of accomplished reports to KMITS on a monthly basis (Annex D.1 and D.2).
 - iii. Monitoring of cybersecurity incidents in the region; and submission of accomplished report to KMITS on a monthly basis (Annex E)
 - c. Assist the CMT in the implementation of the requirements for the Province/City Wide Health System (P/CWHS) Characteristic on Info1: Information Management System (IMS);
 - i. Conduct refresher training of the UHC IS;
 - ii. Coordinate with the UHC IS for the compliance of the requirements from the preparatory to functional level for Info1: Information Management System (IMS);
 - iii. Coordinate with the UHC IS focal for the compliance with the reporting timelines set by the DOH-Bureau of Local Health System Development, and provide assistance in the validation of the submitted reports;
 - iv. Facilitate the provision of technical assistance for the UHC IS.
 - d. Assist in the administration of standards conformance and interoperability validation (SCIV), and assessment and evaluation of third-party ICT service providers within the region;
 - e. Participate in technical trainings and capacity building activities conducted by the DOH-KMITS;
 - f. Develop, enhance, and maintain information systems as needed by their respective CHDs but shall ensure coordination with KMITS prior.
 - g. Assist the CMT in developing work plans for the deployment of EMRs and DOH developed systems to new users and maintenance/site visits to users.
 - h. Form part of the Computer Emergency Response Team with the Computer Maintenance Technologist as per DOH Department Order 2023-0169: "DOH Cybersecurity Framework" and has the following;
 - Assist in the implementation of DOH CSF in their respective CHD
 - Monitor the compliance with the DOH CSF policies, standards, guidelines, and procedures; relevant issuances from the Department of Information Communications Technology, Cybercrime Investigation and Coordinating Center,

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- and National Privacy Commission
 - Ensure proper security incident and data breach management in the CHD preparation and submission to relevant authorities of reports and other documentation concerning security incidents within the prescribed period (Annex E.1 and E.2)
 - Cooperate, coordinate, and seek the advice of the DOH-CERT-PH and other relevant authorities regarding matters concerning cybersecurity; and perform other duties and tasks that may be assigned by the Head of the DOH unit that will strengthen the unit's/units' cybersecurity.
 - i. Assist the CMT in the maintenance of the CHDs servers/HCI; and submission of report to KMITS on a monthly basis (Annex F)
 - j. Serve as the sub-administrator of the National Health Facility Registry (NHFR) that shall approve/disapprove account registration and provide technical support like system walkthrough and navigation to Regulation, Licensing Enforcement Division and Health Facility Development Units for provision of support in validating the requests for inclusion/updating/deletion of health facilities in the system.
 - k. Execute other tasks that may be assigned by the CMT/immediate supervisor and DOH-KMITS.
2. All hired CP I covered under these guidelines shall have the following roles and responsibilities:
- a. Capacitate health facilities and provide technical support to the implementation of the DOH developed and maintained information systems such as but not limited to: iHOMIS, iClinicsys Synchronized Electronic Immunization Repository, Electronic Logistics Management Information System and other applications and services within their respective region;
 - b. Conduct regular site visits, troubleshooting, and checkpoint monitoring all implementing health facilities within the region; and provide technical assistance to all implementing/reporting health facilities, including responding to inquiries/clarifications, and updating FAQs based on encounters during system implementation;
 - c. Assist the CMT III and CP II in providing technical assistance to UHC IS for P/CWHS Characteristic on Info1: IMS;
 - d. Encode/Update/Delete profiles of health facilities in the NHFR.
 - e. Assist the CMT and CP II in the accomplishment of reports indicated in the annexes.
 - f. Assist the CMT in the maintenance of internet connectivity, network, server/Hyper Converged Infrastructure, hardware and software maintenance of the designated CHD;
 - g. Execute other tasks that may be assigned by the CMT/immediate supervisor and DOH-KMITS.
3. All hired CP I and II shall submit a copy of monthly **Accomplishment Report** to sites.google.com/cpreports following the prescribed schedules below:
- a. **An initial report must be submitted on April 15 2024 succeeding reports are required to be submitted on the second working day every three months.**
 - Functional EMR Inventory Report (Annex C.1)
 - Telemedicine Inventory Report (Annex D.1)
 - b. **Monthly Submission every 2nd working day of the following month.**
 - EMR and DOH Information Systems Technical Support Monthly Report (Annex B)
 - Functional EMR Implementation Status Report (Annex C.2)
 - Telemedicine Implementation Status Report (Annex D.2)
 - Monthly Security Incident Report (Annex E.2)
 - Monthly Report on the Monitoring and Maintenance of CHD Infrastructure (Annex F) iHOMIS and iClinicsys Implementation (Annex G)

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Annex C. EMR and DOH Information Systems Technical Support Monthly Report

Reporting Month and Year:				
Support to:	Status			
	Total No. of Requests	Total Resolved	% Accomplishment (total resolved / total requests * 100)	Top 5 Most Common Types of Technical Support Provided
iClinicsys				
iHOMIS				
iHOMIS Plus				
Synchronized Electronic Immunization Repository				
Electronic Logistics Information System				
National Health Facility Registry				
Internet Connectivity and Infrastructure				
Corrective Maintenance of ICT equipment				
Preventive Maintenance of ICT equipment				
Technical Assistance on ICT Network and Communications				
Technical Assistance on ICT Accounts (if applicable) - ICT Accounts - Email - ICT Accounts - WIFI				

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Reporting Month and Year:				
Support to:	Status			
	Total No. of Requests	Total Resolved	% Accomplishment (total resolved / total requests * 100)	Top 5 Most Common Types of Technical Support Provided
iClinicsys				
- ICT Accounts - AD				
Total				

**Add rows as necessary for additional DOH developed information system*


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Annex C.1 Functional EMR Inventory Report

Reporting Month and Year:											
#	Health facility Name	Health Facility Type (Hospital/Infirmary/RHU)	Hospital Level (if Applicable)	Province	City	Barangay	Name of System	Name of Third Party EMR Provider (if Applicable)	Status of Functionality ²	Contact Person of the Health Facility (for Coordination)	Contact Number

² A functional EMR system refers to an EMR system that has passed the software standards conformance and interoperability validation (SCIV) of the DOH, PhilHealth, and DICT as to the minimum scope, standards, and level of functionality relative to: (a) the processing and submission of mandatory health and health-related data and reports at the local and national level; and (b) conduct of electronic coordination of referrals and secure sharing/exchange of medical/health records.

2. The overall P/CWHS performance for implementation of a functional EMR system among health facilities will be measured using data from these health facilities as the initial priority areas for implementation.


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Annex C.2 Functional EMR Implementation Status Report

Reporting Month and Year:									
Health Facility/Office (HF/O)	Total # of LGU-owned HF/O	Total # of LGU-owned/ HF/O Implementing a Functional EMR System	% ³	Total # of DOH-retained HF/O	Total # of DOH-retained HF/O Implementing a Functional EMR System	%	Total # of Private-owned HF/O	Total # of Private-owned HF/O Implementing a Functional EMR System	%
Health Offices (for providing health services)									
1. Provincial Health Office									
2. City Health Office									
3. Municipal Health Office									
Sub-Total									
B. Hospitals									
1. Apex Hospital									
2. Level 3 Hospital									
3. Level 2 Hospital									
4. Level 1 Hospital									
Sub-Total									

³ To get the percentage: $n/d \times 100$. numerator = total number of facilities with functional emr / total number health facilities

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C. Infirmary									
Infirmary									
D. Primary Care Facilities									
1. Rural Health Units									
2. Barangay Health Stations									
Sub-Total									
Grand-Total									


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Annex D.1 Telemedicine Monthly Inventory Report

Reporting Coverage:											
#	Health facility Name	Health Facility Type (Hospital/Infirmary/RHU)	Hospital Level (if Applicable)	Address	Province	City	Barangay	Name of System	Name of Third Party Telemedicine Provider (if Applicable)	Contact Person of the Health Facility (for Coordination)	Contact Number

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Annex D.2 Telemedicine Implementation Status Report

Reporting Coverage:								
Health Facility/Office (HF/O)	Total # of HF/O	Total # of HF/O Providing Tele-medicine Services	Telemedicine Consultations Attended			Medical Interventions Provided		
			Total # of Tele-medicine Consults	COVID-19-related	Non-COVID-19-related	Medical Diagnosis, Health Advice, and Counseling	Referral	Issued Electronic Prescription
Health Offices (for providing telemedicine services)								
1. Provincial Health Office								
2. City Health Office								
3. Municipal Health Office								
Sub-Total								
B. Hospitals								
1. Apex Hospital								
2. Level 3 Hospital								
3. Level 2 Hospital								

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4. Level 1 Hospital								
Sub-Total								
C. Infirmary								
Infirmary								
D. Primary Care Facilities								
18. Rural Health Units								
29. Barangay Health Stations								
Sub-Total								
Grand-Total								


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Annex E.1 Cybersecurity Incident Reporting Form

SECURITY INCIDENT REPORTING FORM

Type or Report: Initial / Updated Findings / Final

Incident No: _____

Overview of the Incident	
Date and Time of the Incident:	Date and Time of Report:
Description of the Incident (<i>Brief description of the incident. What was not working?</i>)	
Nature of Incident (<i>What was the initial observation/issue observed. Please Check all that apply.</i>)	
☐ Hardware (Procured / CPE)	☐ Internet / Intranet / Connectivity-Related
☐ Software / Database	☐ Cybersecurity Incident
☐ Website / Web Application	☐ Others:
Type of Incident	
☐ Account Compromise (e.g., lost password)	☐ Social Engineering (e.g., phishing, scams)
☐ Denial of service (including distributed)	☐ Technical vulnerability (e.g., 0-day attacks)
☐ Malicious code (e.g., virus, worm, Trojan)	☐ Theft/loss of equipment or media
☐ Misuse of systems (e.g., acceptable use)	☐ Unauthorized access (e.g., systems, devices)
☐ Reconnaissance (e.g., scanning, probing)	☐ Unknown/Other (Please describe below)
Description of Incident:	

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Scope of Incident (Check one)	
☐ Critical (e.g., affects public safety or state-wide information resources) ☐ High (e.g., affects departments entire network or critical business or mission systems) ☐ Medium (e.g., affects departments network infrastructure, servers, or admin accounts) ☐ Low (e.g., affects departments workstations or user accounts only) ☐ Unknown/Other (Please describe below)	
Estimated quantity of systems affected:	
Estimated quantity of users affected:	
Third-parties involved or affected: (e.g., vendors, contractors, partners)	
Additional scope information:	
Impact of the Incident (Check all that apply)	
☐ Loss of access to services	☐ Propagation to other networks
☐ Loss of productivity	☐ Unauthorized disclosure of data/information
☐ Loss of reputation	☐ Unauthorized modification of data/information
☐ Loss of revenue	☐ Unknown/Other (Please describe below)
Estimated total cost incurred: (e.g., cost to contain incident, restore systems, notify data owners)	
Additional impact information:	
Sensitivity of Affected Data/Information (Provide as much detail as possible)	
☐ Confidential/Sensitive data/information	☐ Personally identifiable information (PII)
☐ Non-sensitive data/information	☐ Intellectual property/copyrighted data/info
☐ Publicly available data/information	☐ Critical infrastructure/Key sources
☐ Financial data/information	☐ Unknown/Other (Please describe below)
Quantity of data/information affected: (e.g., file sizes, number of records)	
Additional affected data information:	

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Systems Affected by Incident (Provide as much detail as possible)	
Attack sources (e.g., address, port):	
Attack destinations (e.g., address, port):	
IP addresses of affected systems:	
Domain names of affected systems:	
Primary functions of affected systems: (e.g., web server, domain controller)	
Operating systems of affected systems: (e.g., version, service pack, configuration)	
Patch level of affected systems: (e.g., latest patches loaded, hotfixes)	
Security software loaded on affected systems: (e.g., anti-virus, anti-spyware, firewall, versions, date of latest definitions)	
Physical location of affected systems: (e.g., state, city, building, room, desk)	
Additional system details:	
Users affected by Incident (Provide as much detail as possible)	
Names and job titles of affected users:	
System access levels or rights of affected users: (e.g., regular user, domain administrator, root)	
Additional user details:	
Timeline of Incident (Provide as much detail as possible)	
a. Date and time when College/Department first detected, discovered, or was notified about the incident: b. Date and time when the actual incident occurred: (estimation if exact date and time unknown)	


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c. Date and time when the incident was contained, or when all affected systems or functions were restored: (use whichever date and time is later)	
Elapsed time between the incident and discovery: (e.g., difference between a. and b. above)	
Elapsed time between the discovery and restoration: (e.g., difference between a. and c. above)	
Details incident timeline:	
Remediation of Incident (Provide as much detail as possible)	
Actions taken by College/Dept. to identify affected resources:	
Actions taken by College/Dept. to remediate incident:	
Actions planned by College/Dept. to prevent similar incidents:	
Additional Remediation details:	


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Annex E.2 Monthly Security Incident Report

Health Facility/Office (HF/O)	Personal Data Breach, Mandatory Notification (A)	Personal Data Breach, not covered by mandatory notification requirements (B)	Other Security Incidents (C)	Total # of Security Incidents (D = A+B+C)
A. Health Offices				
1. Provincial Health Office				
2. City Health Office				
3. Municipal Health Office				
Sub-Total				
B. Hospitals				
1. Apex Hospital				
2. Level 3 Hospital				
3. Level 2 Hospital				
4. Level 1 Hospital				
Sub-Total				
C. Infirmary				
Infirmary				
D. Primary Care Facilities				
1. Rural Health Units				
2. Barangay Health Stations				
Sub-Total				
Grand-Total				

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Annex F. Monthly Report on the Monitoring and Maintenance of CHD Infrastructure

Reporting Month:	
Service Uptime for the month (No. of hours Up / total number of hours in a month x 100)	

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Annex G: iHOMIS and iClinicsys Implementation - Can be downloaded at sites.google.com/cpreports



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Tayag Enrique
Amar



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